

Recognise the emergency pattern

Sudden ocular pain with haloes, vomiting and a fixed mid-dilated pupil is a high-risk pressure presentation.

Symptoms and matching signs

Clinical clue	Why it matters
Pain, headache and vomiting	Can accompany a marked acute IOP rise
Blur and coloured haloes	Corneal epithelial oedema scatters light
Fixed mid-dilated pupil	Raised pressure may cause iris ischaemia
Shallow chamber and high IOP	Support angle obstruction in a predisposed eye

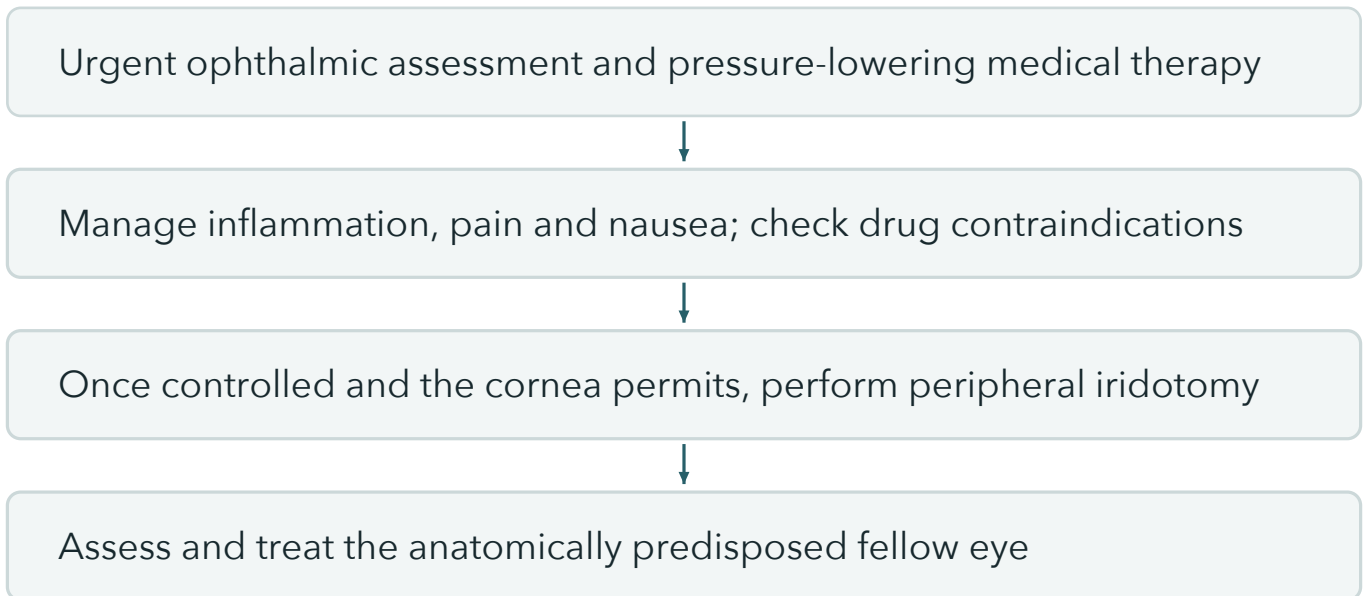
Trace the attack

In pupillary block, iris-lens contact impedes aqueous passage through the pupil. Posterior chamber pressure rises relative to the anterior chamber, bowing the peripheral iris forward onto the trabecular meshwork. Older age, hypermetropia and a short shallow eye increase anatomical susceptibility. Pharmacological mydriasis can precipitate closure. The combined findings require urgent ophthalmic treatment; a headache-dominant history should not distract from examining the painful eye.

Break the attack, then bypass the block

Medical pressure lowering protects the nerve while preparing for definitive treatment of pupillary block.

Urgent care sequence



Understand the laser purpose

Iridotomy creates another route from posterior to anterior chamber, equalising pressure across the iris. It is different from trabeculoplasty, which targets trabecular outflow. Avoid using atropine or another mydriatic in an active pupillary-block attack. Timolol is unsuitable in asthma or significant bradycardia. Residual high pressure after iridotomy needs further evaluation because mechanisms or established damage can remain; lens contribution and additional glaucoma intervention may need consideration.